

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°137/MVE-BDBV/09/28/2026



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HIGHLIGHTS (last 24 hours)

- As of September 28, 55 new confirmed cases of Ebola Virus Disease (EVD) were reported, including 17 deaths. Community transmission has been reported. The confirmed cases were distributed across the provinces of Ituri (36 cases), North Kivu (12 cases), Haut-Uélé (5 cases), and Tshopo (2 cases). In addition, 13 deaths were recorded in Ebola Treatment Centers (ETCs).
- A total of 380 new samples were analyzed; 55 of these samples were found to be positive for the Bundibugyo virus, representing a positivity rate of 14.5%.
- No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 8,171	ACCUMULATIVE DEATH CONFIRMED 3,954	LETHALITY 48.4%	PATIENTS IN ISOLATION/CTE 759	ACCUMULATIVE HEALED 2,105	TRACKING RATES CONTACTS (Of the Day) 82.0%
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Affected provinces and affected health zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



**63 out of 167
health zones affected
(37.7%)**

Summary of key points from the last 24 hours

Fifty-five (55) new confirmed cases of Ebola Virus Disease (EVD), including **17 community deaths**, have been recorded in the last 24 hours. The provinces reporting these cases are Ituri (36 cases), North Kivu (12 cases), Haut-Uélé (5 cases) and Tshopo (2 cases).

In addition, **13 deaths were recorded in the CTEs** (11 in Ituri, 1 in North Kivu and 1 in Haut-Uélé), which brings to 30 the total number of confirmed deaths for the period considered.

At the end of the same day, 17 patients (**11 in Ituri, 4 in Tshopo and 2 in Bas-Uélé**) were declared cured after obtaining two negative results for Ebola virus disease.

As of **September 28, 2026**, the DRC has recorded **8,171 confirmed cases and 3,954 deaths**. (Case fatality rate: **48.4 %**). Ituri remains the epicenter of the epidemic with **76.1%** of cumulative confirmed cases reported and **65.5%** of new confirmed cases. In contrast, North Kivu province has a case fatality rate higher than the national average, at **59.4%**.

In total, **63 out of 167 health zones (37.7%)**, spread across **seven provinces**, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 28, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	36	6218	2,868	46.1%	28/36 (77.8%)
North Kivu	12	1553	922	59.4%	16/34 (47.1%)
Haut-Uélé	5	340	142	41.8%	7/13 (53.8%)
Tshopo	2	45	16	35.6%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	10	4	40.0%	3/11 (27.3%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
Total	55	8,171	3,954	48.4%	63/167 (37.7%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 28, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6218	2868	46.1%	36	9	11	20
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	202	46	22.8%				0
Boga	2	2	100.0%				0
Bunia	1714	557	32.5%	9	4	3	7
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	82	31	37.8%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	35	12	34.3%				0
Komanda	168	101	60.1%	1	0	0	0
Lita	284	143	50.4%	1	1	1	2
Logo	13	5	38.5%				0
Lolwa	33	8	24.2%				0
Mahagi	4	1	25.0%				0
Mambasa	29	11	37.9%				0
Mandima	131	71	54.2%	11	4	0	4
Mangala	350	153	43.7%	4	0	0	0
Mongbwalu	700	330	47.1%	2	0	2	2
Nia-Nia	295	161	54.6%	2	0	2	2
Nizi	775	279	36.0%	3	0	1	1
Nyankunde	128	37	28.9%				0
Rimba	12	4	33.3%				0
Rwampara	1082	365	33.7%	3	0	2	2

Tchomia	76	25	32.9%				0
To ventilate *	N / A	493	N / A				0
North Kivu	1553	922	59.4%	12	7	1	8
Blessed	343	228	66.5%	6	6	0	6
Good	18	7	38.9%				0
Butembo	258	167	64.7%	1	0	1	1
Goma	1	0	0.0%				0
Kalunguta	54	18	33.3%	1	0	0	0
Katwa	559	355	63.5%				0
Kayna	1	0	0.0%				0
Kyondo	33	20	60.6%				0
Lubero	21	11	52.4%				0
Mabalako	32	11	34.4%	1	0	0	0
Manguredjipa	13	5	38.5%				0
Masereka	28	9	32.1%				0
Musienene	123	66	53.7%	1	0	0	0
Mutwanga	12	2	16.7%				0
Oicha	26	14	53.8%				0
Vuhovi	31	9	29.0%	2	1	0	1
Haut-Uélé	340	142	41.8%	5	1	1	2
Boma Mangbetu	39	17	43.6%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	102	44	43.1%	1	1	0	1
Pawa	83	38	45.8%	3	0	1	1
Rungu	2	1	50.0%				0
Wamba	110	40	36.4%	1	0	0	0
Tshopo	45	16	35.6%	2	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	24	10	41.7%	1	0	0	0
Mangobo	6	3	50.0%	1	0	0	0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	10	4	40.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Ganga	3	2	66.7%				0
South Ubangi	2	1	50.0%	0	0	0	0
Bulu	2	1	50.0%				0
Total	8171	3954	48.4%	55	17	13	30

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for September 28, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	28	6	34	17	6	9	0	23	15
Bas Uélé	150	3	153	10	3	140	0	13	0
Ituri	1123	77	1200	128	74	995	0	198	123
North Kivu	966	39	1005	131	39	811	0	170	101
South Ubangi	56	0	56	3	0	0	0	3	1
Tshopo	173	0	173	9	0	164	0	9	1
Total General	2,496	125	2 621	298	122	2,119	0	416	241

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

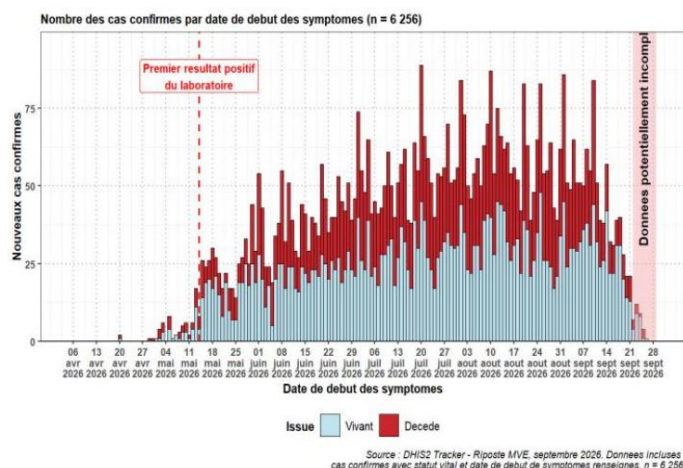
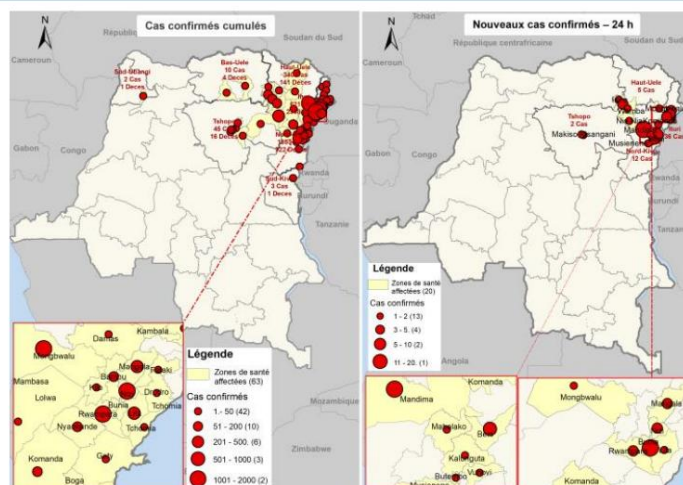


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

During the course of the day on September 28, 2026, **2,621** alerts were raised, **2,539** (96.9%) were verified, of which **420** (16.5%) were validated as suspected cases and **416** (99.0%) were investigated. Thus, 241 suspected cases were transferred to CT/CTE (Table 3).

Of the **30,159** contacts currently being monitored, **24,717** were seen in the last 24 hours, representing a follow-up rate of **82.0%**, higher than the previous day's rate (**75.9%**) but still below the required threshold (95.0%). In South Ubangi, this rate was 93.9% (92/98), 85.9% (159/185) in Bas-Uélé, 84.4% (10,534/12,476) in Ituri, 83.2% (634/762) in Tshopo, 80.1% (11,921/14,876) in North Kivu, and 78.1% (1,377/1,762) in Haut-Uélé.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Eight (8) alerts were notified to the PoE/PoC during the day, including five (5) in Ituri and three (3) in North Kivu. Three live alerts were validated, two of which were transferred (transfer rate of 66.7%), and four intercepted bodies were transported for swab.

Table 4. Status of key PoE/PoC indicators by province, as of September 28, 2026

Indicators	Ituri	North Kivu	Tshopo	Overall
Report completeness	95.0% (57/60)	90.5% (19/21)	44.4% (8/18)	84.8% (84/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	154,875	89,299	7093	251,267
% of travelers screened	96.8%	97.3%	96.3%	97.0%
% of travelers who washed their hands	96.7%	97.3%	96.5%	96.9%
% of travelers made aware	98.4%	97.3%	100.0%	98.1%
Number of alerts notified	5	3	0	8
Number of validated live alerts (suspected cases)	2	1	0	3
Number of bodies recovered today	3	1	0	4
% of suspected cases transferred to CTE/CT	50.0%	100.0%	N / A	66.7%
% of lifeless bodies swabbed/secured	100.0%	100.0%	N / A	100.0%
% of suspected cases investigated in 4 h	100.0%	100.0%	N / A	100.0%

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 36 new positive results** (27 living and 9 deaths) out of 200 samples received and analyzed (**positivity: 18.0%**).
- **North Kivu: 12 new positive results** (5 living and 7 dead) out of 140 samples received and analyzed (**positivity: 8.6%**).
- **Haut-Uélé: 5 new positive results** (4 living and 1 death) out of 20 samples received and analyzed (**positivity: 20.0%**).
- **Tshopo: 2 new positive results** (2 live) out of 10 samples received and analyzed (**positivity: 25.0%**).
- **Bas Uélé: 10 samples received and analyzed** (1 live and 1 deceased), all returned negatives.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 14 out of 33 rings were opened (42.4%), 32 out of 37 households were decontaminated (86.5%), and 14 out of 19 health facilities were decontaminated (73.7%). Of 82 EDS alerts (including 8 postponements), 58 EDS were carried out (70.7%) and 58 bodies were swabbed.
- **In North Kivu,** 20 out of 27 rings were opened (74.1%), 7 out of 27 households were decontaminated (25.9%), and 19 out of 24 health establishments were decontaminated (79.2%). In total, 184 health facilities were monitored, 582 service providers were briefed, and 38 health impact assessments (HIAs) were carried out out of 56 alerts.
- **Haut-Uélé :** 2 out of 4 rings were opened (50.0%), 6 out of 6 households were decontaminated (100%), and 3 out of 3 ESSs were decontaminated (100%). Out of 5 EDS alerts, 2 EDSs were carried out and 2 bodies were swabbed.

- **Tshopo** : 18 health establishments were monitored in 6 health zones and 186 service providers were briefed. Cumulatively, 25 out of 29 households (86.2%) and 33 out of 33 health establishments (100%) that had hosted a confirmed case were decontaminated within 48 hours.
- **Bas-Uélé** : 5 ESS followed in Viadana and Buta, start in Ganga of a PCI/WASH training of 70 participants (FHI 360); 2 bodies swabbed in Buta and Ganga and one failure in Poko.

2.4.2. Challenges

In **Ituri**, the opening of infection control rings remains low (42.4%; 14/33), 19 bodies have not been swabbed, and activities are suspended in Nizi due to insecurity. In **North Kivu**, household decontamination remains limited (25.9%; 7/27), no decontaminated household received a kit (0/7), and the timeliness/completeness of infection prevention and control (IPC) reports is 68.8% (11/16 health zones). In **Haut-Uélé**, reporting is incomplete (57.1%; 4/7 health zones); two rings could not be opened in Pawa (Fungula and Gbonzunzu health zones) due to community tensions. In **Tshopo**, only 2.0% of health centers (2/99) obtained an IPC score above 80%. The distribution of PCI/EHA kits to households remains limited to 40.0% (20/50), and the incinerator at the Makiso-Kisangani General Referral Hospital remains out of service. In **Bas-Uélé**, two out of three waste collection centers in Buta are not operational, and acceptance of EDS (Emergency Response Services) remains low.

2.5. Holistic care

2.5.1. Main actions

- In **Ituri**, 93 new admissions were recorded compared to 80 discharges, including 11 recoveries. In total, 381 patients remained in isolation at the end of the day. The bed occupancy rate was **37.5%** ; 360 patients received hot meals and children under LNPE/SLM were fully covered at the CTE (9/9), in the nursery (9/9) and in the community (33/33); 39 patients are in critical condition.
- In **North Kivu**, 69 new admissions were recorded, compared to 48 discharges, with no recoveries. A total of 366 patients remained hospitalized, including 249 in standardized facilities. The bed occupancy rate was **70.3%**.
- In **Tshopo**, 6 new admissions were recorded. Seven discharges were reported, including 4 recoveries. Nine patients remained in isolation at the end of the day, representing an occupancy rate of **29.0%**.
- In **Bas-Uélé**, 2 new admissions were recorded against 4 discharges including 2 recoveries. Three patients remained in isolation at the end of the day.

2.5.2. Challenges

- In **Ituri**, some CTE/CT facilities have limited capacity and inadequate care is provided. food-related issues remain a concern.
- In **Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.
- In **Tshopo**, the continued presence of confirmed cases in isolation necessitates the maintenance of infection prevention and control measures.

2.6. Vaccination

2.6.1. Main actions

- In **Ituri**, 66 people were vaccinated on September 28 with MSF support, including 34 women and 32 men, with no reported adverse events following immunization (AEFIs). The cumulative total has reached 602 people vaccinated, including 251 women and 351 men. Vaccination activities and research on the efficacy of the ERVEBO vaccine against BDBV-related Ebola virus disease are ongoing.
- In **Bas-Uélé**, 19 people with HIV/AIDS and those with HIV/AIDS were vaccinated in Buta, bringing the provincial total to 1,019 people vaccinated out of a target of 13,202 (7.7%). Compassionate vaccination is being prepared in Aketi and Viadana.
- In **Tshopo**, the vaccine shortage persists. Nationally, 6,897 people have been vaccinated, i.e. 100.3% of the pre-registered target.

2.6.2. Challenges

In **Tshopo**, there was a shortage of the ERVEBO vaccine in a context of strong community demand; the two providers confirmed on September 28 were neither vaccinated nor on PEP.

In **Bas-Uélé**, the UCCs are up and running and the validated micro-plan and vaccination funds are available.

In **Ituri**, the target for PPL/TPL remains to be redefined; no reliable coverage rate is calculated on this basis in the presentation.

2.7. Continuity of Care

2.7.1. Main actions • In Tshopo,

activities on September 28 included the evaluation of the CELPA Medical Center, the development of the recovery plan for the Makiso-Kisangani Medical Center, the validation of the PCSSE work plan for the fourth week of September, and the monitoring of PPLs at the CTM and CTE. Of the 51 staff members at the evaluated facility, 40 are vaccinated; a functional incinerator was noted, but the use of the thermal scanner remains optional.

2.7.2. Challenges

In **Tshopo**, there has been a decrease in the use of services, particularly maternity and cesarean sections, in the assessed ESS; the temporary closure of the DHKis ESS remains proposed in the face of the multiplication of confirmed and suspected cases.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- In **Ituri**, 11,389 home visits reached 32,663 people; 387 educational talks brought together 2,448 people, 8 community debates reached 351 people, and 37 mass awareness campaigns reached 8,813 people. A total of 304 community alerts, including 94 deaths, were reported; all 41 feedback requests and 6 incidents recorded were resolved within 48 hours.
- In **North Kivu**, CREC supported the training of community relays in Katwa and facilitated, around the latest cases, the listing of 15 contacts in Butembo and 11 in Lubero as well as the decontamination of a health post in Manguredjipa; four reservations remain under negotiation in Butembo.
- In **Tshopo**, 208 home visits reached 1,165 people, 4 educational talks reached 162 people, and 12 mass awareness sessions reached 15,286 people affected). The 27 community alerts notified (22 in Makiso-Kisangani and 5 in Tshopo) have been processed, the 2 feedback issues resolved and the 2 refusals lifted.
- In **Bas-Uélé**, 143 home visits and 3 mass awareness campaigns reached 1,859 people in Viadana and Ganga. In Titule, a rumor about student vaccinations led parents to withdraw their children from school; an emergency meeting and an appeal on Bima radio followed.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **North Kivu**, 16 confirmed cases benefited from SMSPS interventions in 5 of the 6 health zones that notified. The teams supported 25 households of separated and orphaned children (100%), announced 103 results, including 17 positive ones, and reintegrated 5 recovered children.
- In **Ituri**, 17 people among the new confirmed cases, 109 confirmed cases currently under follow-up out of 150 (72.7%), 32 new suspected cases out of 93 (34.4%), and 111 suspected cases currently under follow-up out of 120 (92.5%) benefited from SMSPS interventions. The teams supported 459 caregivers and 79 people living with HIV, monitored 16 recoveries out of 23 in the community (69.6%), and announced 62 results, including 17 positive ones.
- In **Tshopo**, teams provided psychological support to 2 confirmed cases under follow-up and carried out psychoeducation for 30 suspects and 147 people; 3 results, all negative, were announced and the family of the chief of the village of Bafwandambo was reintegrated after the recovery of their child.

2.9.2. Challenges

- **In Ituri**, urgently strengthen the capacities of psychologists and psychosocial agents for interventions around cases in Mandima.
- **In Tshopo**, train the psychosocial agents of Wanie-Rukula and Bafwasende, strengthen the means of communication and movement of the pillar, and provide material support to the family of the healed person of Bafwandambo.

2.10. Logistics

2.10.1. Main actions

- **In North Kivu**, 97 packages of IPC supplies were received from the WHO, and medicines and IPC supplies were delivered from the UG-PDSS to Masereka.
- **In Tshopo**, monitoring of the construction of the Bafwasende CTE, support for the mobility of pillars and reception of inputs for compassionate treatment (INRB).
- **In Bas-Uélé**, delivery of chairs, mattresses and PCI supplies and support to the ZS of Ganga and Viadana; continuation of the construction of the transit center of the HGR of Buta.

2.11. Security

2.11.1. Main actions

- **In North Kivu**, the Risk Mitigation and Security pillar provided 100% coverage for EDS teams for 3 burials of confirmed cases in Beni and 3 swabs in the Katwa Health Zone

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- **In Ituri**, 36 staff members (11 women and 25 men) were briefed on the PSEAH and signed the code of conduct; 273 people were sensitized.
- **In North Kivu**, 170 people were sensitized on the PSEAH, 88 in Goma and 82 in Beni; 40 leaflets were distributed.

2.12.2. Challenges

In Haut-Uélé, at the national level, only 2 out of 7 DPS reported their PSEA activities (28.6%) and CBCMs remain largely ineffective.

KEY MESSAGES

- Active and sustained community transmission remains observed in the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo, and Bas-Uélé. Meanwhile, the province of Sud-Ubangi has not reported any confirmed cases in the last 24 hours.
- Progress to be consolidated has been recorded in the field of epidemiological investigations: Almost all (**99.0 %**) of suspects were investigated within 24 hours of their notification.

However, the contact tracing rate is higher compared to the previous day (**82.0 vs 73.9%**).
- Strengthen surveillance, care, infection prevention and control (IPC), logistics, and community engagement, prioritizing increased capacity at Ebola Treatment Centers (ETCs)/Treatment Centers (TCs) in North Kivu, rapid transfer of suspected cases, and isolation of confirmed cases. **In Mangobo (Tshopo)**, two confirmed cases who escaped from the ETC remain reported at home.
- Strengthen preparedness and response to Ebola virus disease B in the river corridor and surrounding areas, particularly in South Ubangi, through active search, contact tracing and availability of inputs. Continue cross-border monitoring with South Sudan.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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